BASELINE → TREATMENT → FOLLOW-UP

Your testing and review plan

A reference cycle is not a personalised calendar. Tests listed as symptom-triggered or individual are not presented as mandatory tests at every visit. No universal imaging or tumour-marker interval is invented.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms	Confirm BRAF V600 and check blood count, kidney and liver function, electrocardiogram (ECG), medicines and skin. Record existing skin lesions and relevant head-and-neck or other cancer assessments so later changes can be compared.	Monthly blood count, kidney and liver tests, and monthly skin review. Repeat ECG when clinically indicated. National Cancer Control Programme (NCCP) also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan.	If your temperature is 38°C or higher, stop dabrafenib and contact the oncology team immediately under the current product information; do not restart on your own. Also call for shaking chills, dizziness, reduced urine, severe abdominal pain, eye pain or changed vision, or a blistering or spreading rash. Infection and medicine-related fever both need consideration.	Continue skin surveillance for up to six months after stopping, as specified by NCCP. Keep the agreed review for other new lesions and the individual response-imaging plan. No fixed tumour-marker schedule is assumed.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00237 v6: current regimen source · EMA Tafinlar product information, updated 19 May 2026

THE WHOLE TREATMENT JOURNEY

Dabrafenib alone — advanced melanoma: treatment and checks

Advanced melanoma

Dabrafenib twice daily, about 12 hours apart, every day. Take it on an empty stomach; there is no planned tablet-free week.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Safety tests and symptoms NCCP 00237	Confirm BRAF V600 and check blood count, kidney and liver function, ECG, medicines and skin. Record existing skin lesions and relevant head-and-neck or other cancer assessments so later changes can be compared.	Monthly blood count, kidney and liver tests, and monthly skin review. Repeat ECG when clinically indicated. NCCP also describes head-and-neck review every three months and other surveillance according to the individual assessment; this is separate from the cancer-response scan plan.	If your temperature is 38°C or higher, stop dabrafenib and contact the oncology team immediately under the current product information; do not restart on your own. Also call for shaking chills, dizziness, reduced urine, severe abdominal pain, eye pain or changed vision, or a blistering or spreading rash. Infection and medicine-related fever both need consideration.	Continue skin surveillance for up to six months after stopping, as specified by NCCP. Keep the agreed review for other new lesions and the individual response-imaging plan. No fixed tumour-marker schedule is assumed.	Do not restart held treatment until the team confirms the medicine, dose and timing. Safety tests and unresolved symptoms are reviewed first.
Scans and tumour results NCCP 00237	Keep the pathology, stage and relevant biomarker report.	Response scans follow the individual disease plan, separate from safety bloods.	Report new symptoms rather than waiting for a scan.	Ask which scan, its planned date and who will explain the result.	A treatment hold or stop may change the next scan or follow-up date.
Written plan and contacts NCCP 00237	Record actual oncology and out-of-hours numbers; examples are not working contact details.	Bring your medicine list and record the dose actually given or taken.	Keep the wallet card and written urgent-symptom plan available.	Ask for a printed updated plan when treatment changes.	Keep the last-dose date and any steroid, hormone or other follow-up instructions.

What each check is for

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Ongoing oral treatment

28-day reference cycle

To control advanced BRAF V600-mutated melanoma using dabrafenib alone, with regular review of benefit, fever and other side effects.

Ask / **act:** Confirm the current phase, prescription, tests and support medicines.

Sources: NCCP 00237

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.42